[DATE], [TIME] Markandayev, Jason ([MRN]) [DATE] Encounter Date:

Markandaye

MRN:

Mike Boykin N, MD

Physician
Specialty: Nephrology

Patient: [NAME]:

Date of Birth:

Medical Record#

Date:

Progress Notes Encounter Date: [DATE]
Signed

Renal Medicine Associates Office Visit

Jason Markandayev

[DATE], 70 y.o. Male

987456

[DATE]

History of Present Illness

Jason Markandayev is a 70 y.o. male with past medical history significant for biopsy-proven IgA nephropathy since

July 2021, persistent microscopic and occasional gross hematuria, and asthma. He is currently on losartan

He has not had any episodes of gross hematuria or URIs. He feels well. He does not have ankle swelling.

Home blood pressures are reviewed and are satisfactory.

The following portions of the patient's chart were reviewed in this encounter and updated as appropriate:
Tobacco I Allergies I Meds I

Past Medical History:

Diagnosis Date

- Asthma

goes to Breathe America

Chronic prostatitis

Contact dermatitis and other eczema, unspecified cause

Essential hypertension

Gastroesophageal reflux disease

Hyperkalemia

lgA nephropathy

Kidney biopsy: 7/2/27

Proteinuria

[DATE]

[DATE]

Past Surgical History:

Markandayev, Jason ([MRN]) Printed at [DATE] 1/7

[DATE], [TIME] Markandayev, Jason **(MRN** 987456) [DATE] Encounter Date:

Procedure Laterality Date

NASAL SEPTUM SURGERY

OTHER SURGICAL HISTORY

VASECTOMY

2004

- VASECTOMY 06/2019

Family History

Problem Relation Age of Onset

Mother

Father

Father

Father

Sister

Brother

Maternal Grandmother

Maternal Grandfather

Maternal Grandfather

Maternal Grandfather

Paternal Grandmother

Paternal Grandfather

Paternal Grandfather

Paternal Grandfather

Mother's Sister

Mother's Brother

Father's Sister

Father's Brother

Never

Never

Yes

1.0 - 2.0 standard drink of alcohol

1 - 2 Standard drinks or equivalent per week

Hypertension

Hypertension

Ankylosing spondylitis

Hyperlipidemia

No Known Problems

No Known Problems

No Known Problems

Melanoma

Dementia

Stroke

No Known Problems

Diabetes

Stroke

Dementia

No Known Problems

No Known Problems

No Known Problems

No Known Problems

Social History

Tobacco Use

Smoking status:

Smokeless tobacco:

Substance Use Topics

- Alcohol use:

Alcohol/week:

Types:

Review of Systems

Constitutional: Negative for chills, fever and weight loss.

HENT: Negative for congestion and hoarse voice.

Respiratory: Negative for cough and shortness of breath.

Cardiovascular: Negative for chest pain, palpitations and leg swelling.

Gastrointestinal: Negative for abdominal pain, nausea and vomiting.

Genitourinary: Negative for dysuria, frequency, hematuria and urgency.

Musculoskeletal: Negative for joint pain and neck pain.

Skin: Negative for itching.

Neurological: Negative for dizziness, tremors, focal weakness and numbness.

Endo/Heme/Allergies: Does not bruise/bleed easily.

Psychiatric/Behavioral: Negative for depression and substance abuse.

Current Outpatient Medications

Medication Sig Dispense Refill

- albuterol HFA (PROVENTIL

- albuterol HFA (PROVENTIL Inhale 2 puffs every

HFA;[NAME]) 108 (90 4 (four) hours

Base) MCG/ACT inhaler (Patient: [NAME]: Inhale 2

puffs every 4 (four)

hours if needed for

wheezing or

shortness of breath)

- budesonide-formoterol

HFA;[NAME]) 108 (90

Base) MCG/ACT inhaler

(SYMBICORT) 160-4.5 MCG/ACT

inhaler

Inhale 2 puffs 2 (two)

times a day

- cetirizine (ZyrTEC) 10 MG tablet Take 1 tablet by

mouth 1 (one) time

each day

- losartan (COZAAR) 50 MG Take 1 tablet (SO mg

tablet

Take 1 tablet (SO mg

total) by mouth 1
(one) time each day

predniSONE 5 MG tablet Take 4 tablets (20
mg total) by mouth

1 (one) time each

day if needed (See

instructions) for up

to 14 days Taper to

15 mg after 3 days

and taper by 5 mg

every 3 days until

finished

No current facility-administered medications for this visit.

Allergies

30 tablet 18

70 tablet 1

llergen Reactions

Anaphylaxis and Other (see comments)

Shellfish Allergy

Nsaids

Contraindicated b/c of lgA nephrologist

Shellfish Protein-Containing Drug Products Other (see comments)

[DATE], [TIME]

Physical Exam

Vitals:

[DATE] 0824

BP: 133/92

BP Location: Left upper arm
Patient: [NAME]:

Pulse: 72

Weight: 157 lb (71.2 kg)
Height: 5' 8" (1.727 m)

133/92

Left upper arm
Sitting

72

157 lb (71.2 kg)

5' 8" (1.727 m)

Vitals reviewed.

Constitutional: He is oriented to person, place, and time. He does not appear ill.

HEENT:

Head: Normocephalic.

Right Ear: Hearing normal.

Left Ear: Hearing normal.

Nose: Nose normal. Mouth/Throat: Oropharynx is clear and moist.

Eyes: Conjunctivae and lids are normal. Pupils are equal, round, and reactive to light. No scleral icterus.

Neck: Trachea normal. No JVD present. No thyroid mass and no thyromegaly present.

Cardiovascular: Normal rate, regular rhythm and normal pulses. Exam reveals no friction rub.

No murmur heard.He exhibits no edema.

Pulmonary/Chest: Effort normal and breath sounds normal. No respiratory distress. He has no wheezes.

He has no rales.

Abdominal: Soft. Bowel sounds are normal. There is no abdominal tenderness. No hernia.

Musculoskeletal: Normal range of motion. He exhibits no deformity.

Neurological: He is alert and oriented to person, place, and time.

Skin: Skin is warm and dry. Capillary refill takes less than 2 seconds. No rash noted. No erythema.

Psychiatric: He has a normal mood and affect. His behavior is normal. Judgment normal.

Labs

Chemistry

[DATE], [TIME] Markandayev, Jason ([MRN]) [DATE] Encounter Date:

HEMATOCRIT % 47 47 48 49

HEMOGLOBIN gm/dl 16.1 15.3 15.9 16.7

[NAME]

260

196

x10E3/ul

219

220

< > = values in this interval not displayed.

Bone Mineral

Urine

No lab exists for component: "[NAME]"

Assessment&Plan

1.Biopsy-proven IgA nephropathy

- Status post kidney biopsy in July 2021.

GFR remains satisfactory over 100 ml/min.

He had presented with mild hyperkalemia following increasing losartan to 50 mg daily. He should continue that dose. We discussed high potassium foods. He had since discontinued Gatorade.

- Continue to monitor closely. Recheck serum potassium in 3 months and again prior to the next visit.
- He should maintain an adequate fluid intake of 2.5 L or less.
- He should maintain low-sodium intake of 2000 mg or less.
- He does not have hypertension. Home blood pressures are reviewed and are satisfactory. Systolics have ranged 180.

Serum potassium has normalized and he has reduced Gatorade intake.

At this time immunosuppressive therapy is not indicated.

He should continue to ensure adequate fluid intake of 2.5 L daily.

He should maintain a low-sodium intake of 2000 mg or less.

- He should continue to monitor blood pressure and record.

2. IgA nephropathy

- Proteinuria is less than 200 mg/g. He only has intermittent gross hematuria associated with upper

respiratory infection.

Markandayev, Jason ([MRN]) [DATE] Encounter Date:

- Burst steroid doses prescribed for respiratory infections.
- There is no indication for prolonged immunosuppressive therapy at this time. We will continue to monitor closely. Discussed overall treatment plan with the patient.
- She should continue to monitor blood pressure on record. Blood pressure currently very satisfactory.

1.

2.

Primary IgA nephropathy

Benign essential hypertension

Orders Placed This Encounter

CBC

PTH,Intact

Renal Function Panel

Uric Acid

Urine Albumin/ Creatinine Ratio

Potassium

predniSONE 5 MG tablet

Return in about 6 months (around [DATE]).

No LOS data to display

Mark Bentley, MD

Electronically signed by Mike Boykin N, MD at [TIME]

Office Visit on [DATE] _Note shared with patient_

Additional Documentation

Vitals: BP 133/92 (BP Location: Left upper arm, Patient: [NAME]: Sitting) Pulse 72 Ht 5' 8" (1.727 m)

Wt 157 lb (71.2 kg) BMI 23.87 kg/m [2] BSA 1.85 m [2]

Chart Review Routing History

No routing history on file.

Orders Placed

CBC

Potassium

PTH, Intact

Renal Function Panel

Uric Acid

Visit Diagnoses

Primary:**Primary IgA nephropathy**N02.8
Benign essential hypertension 180